

geha-coverage-policy-paclitaxel-protein-bound.pdf

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Paclitaxel Protein Bound: Abraxane

Original Effective Date: 5/2/2022

Revision Date(s): 9/2021, 11/2022, 12/2022, 7/2023, 12/2023, 4/8/2024, 5/23/2024, 9/4/2024, 11/4/2024, 5/27/2025, 7/1/2026

Review Date: 7/1/2026

Policy type: Medical Necessity

Line of Business: All G.E.H.A Medical Plans

Authorizations are issued for 6 (six) months, unless the ordering physician requests a different timespan or the patients' unique circumstance or condition supports the medical necessity for a different authorization timeframe. Reauthorization requests are reviewed for efficacy, safety and tolerability.

Preference	Requires Prior Auth	Drug Name	HCPCS Code	Description
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Preferred	No	Docetaxel (Taxotere)	J9171	Injection, docetaxel, 1 mg
Preferred	No	Docetaxel (Hospira 505(b)(2))	J9232	Injection, docetaxel (Hospira), not therapeutically equivalent to J9171, 1 mg
Preferred	Yes	Paclitaxel (Taxol)	J9267	Injection, paclitaxel, 1 mg
Non- Preferred**	Yes	Paclitaxel Protein- Bound (Abraxane)	J9264	Injection, paclitaxel protein-bound particles, 1 mg

**Does not apply in the setting of Pancreatic Cancer, Ampullary Adenocarcinoma, and Biliary Tract Cancers.

Members must have documentation of a contraindication, failure, or intolerance to any of the preferred agents (or their 505(b)(2) equivalents) prior to approval of a non-preferred product, when clinically appropriate.

Preferred product requirements apply to new starts only.

Step Therapy applies to all overlapping compendia supported indications/regimens.

Universal Approval Criteria:

Unless otherwise noted, the review criteria used by OncoHealth to determine medical necessity for anticancer treatments and supportive agents include, but is not limited to:

National Comprehensive Cancer Network (NCCN) - use consistent with NCCN recommendations carrying a Category 1 or 2A level of evidence; OR

United States Food and Drug Administration (FDA) labeling - new drugs or regimens (combinations of drugs) consistent with the product labeling; OR

Indications not included in the official FDA labeling or recommended by NCCN (Category 1 or 2A level of evidence) may be considered if determined to be medically necessary per one or more of the following compendia:

O Clinical Pharmacology (Strong For); OR

O Wolters Kluwer Lexi-Drugs (Level A); OR

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*= G.E.H.A

Other uses of drugs and biologics may be considered medically necessary if supported as safe and effective according to peer-reviewed articles from one of the following journals:.

O American Journal of Medicine; Annals of Internal Medicine; Annals of Oncology; Annals of Surgical Oncology; Biology of Blood and Marrow Transplantation; Blood; Bone Marrow Transplantation; British Journal of Cancer; British Journal of Hematology; British Medical Journal; Cancer; Clinical Cancer Research; Drugs; European Journal of Cancer (formerly the European Journal of Cancer and Clinical Oncology); Gynecologic Oncology; International Journal of Radiation, Oncology, Biology, and Physics; The Journal of the American Medical Association; Journal of Clinical Oncology; Journal of the National Cancer Institute; Journal of the National Comprehensive Cancer Network (NCCN); Journal of Urology; Lancet; Lancet Oncology; Leukemia; The New England Journal of Medicine; Radiation Oncology;

O Meeting abstracts and case reports are excluded from consideration;.

Non-standard protocols may be approved based on unique clinical circumstances;

Dose and frequency should be consistent with United States Food and Drug Administration (FDA) labeling, National Comprehensive Cancer Network (NCCN), or indication specific peer-reviewed literature;

In the instance that a request is made for drug(s) that was (were) previously tried (including in the same pharmacologic class or with the same mechanism of action) and such drug(s) was (were) discontinued due to a lack of efficacy the request may be subject to an off-label review for medical necessity unless supported by the NCCN or high quality literature (prospective phase 2 or 3 studies published as full manuscripts in a CMS-supported journal).

Billing

Drug Name	HCPCS Code	Description
Docetaxel (Taxotere)	J9171	Injection, docetaxel, 1 mg
Docetaxel (Hospira 505(b)(2))	J9232	Injection, docetaxel (hospira), not therapeutically equivalent to J9171, 1 mg
Abraxane	J9264	Injection, paclitaxel protein-bound particles, 1 mg
Paclitaxel (Taxol)	J9267	Injection, paclitaxel, 1 mg

References

1. Abraxane [package insert]. Abraxis BioScience, LLC, Summit, NJ. Available at: https://packageinserts.bms.com/pi/pi_abraxane.pdf
2. Abraxane. NCCN Drugs & Biologics Compendium. Available at: https://www.nccn.org/professionals/drug_compendium/content/
3. Paclitaxel Protein-Bound Particles [package insert]. American Regent, Inc., Shirley, NY. Available at: https://americanregent.com/media/3397/ref-2269_paclitaxel-albumin-full-prescribing-informationrev-07-2022.pdf
4. Paclitaxel Protein-Bound Particles [package insert]. Teva Pharmaceuticals, Parsippany, NJ. Available at: https://www.accessdata.fda.gov/drugsatfda_docs/label/2023/216338s000lbl.pdf
5. Paclitaxel, albumin bound. NCCN Drugs & Biologics Compendium. Available at: https://www.nccn.org/professionals/drug_compendium/content/

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10. Referenced with permission from the NCCN Clinical Practice Guidelines in Oncology (NCCN Guidelines). National Comprehensive Cancer Network, Inc. 2026. All rights reserved. Accessed June 5, 2026. To view the most recent and complete version of the guideline, go online to NCCN.org.

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Date	Updates
5/2/2022	Policy approved at P&T
10/14/22	Policy updated to include additional criteria for biliary tract cancers.
11/7/22	Policy approved at P&T to include update from 10/14/22
12/12/22	Updated formatting and incorporated universal criteria.
7/18/23	Policy updated indication specific criteria (Deleted Hepatobiliary indication due to updated NCCN Category 2B recommendation) and to include 505(b)(2) HCPCS
6/1/2024	Policy adopted by GEHA
12/4/24	Updated Step Therapy Table; Removed Indication Specific Universal Criteria; Added J9258;

Updated References

4/8/24	Annual Review. Updated Universal Criteria.
5/23/24	Addition of Docivyx as a Non-Preferred product.
9/4/2024	Update HCPCS Code description for J9172; Delete HCPCS Code J9258
11/4/2024	Update HCPCS Code for 1/1/2025 CMS Updates - J9259 deleted; removed Docivyx from policy
2/1/2025	J9259 removed - code retired 12/31/2024
5/27/2025	Annual review; approved by P&T 5/27/2025
6/5/2025	Document assessed for universal accessibility.
6/5/2026	Reviewed and approved by OH P&T for effective date 7/1/2026 - to include Hospira 505(b)(2) product and CMS permanent HCPCS